



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : ايمان فهيم مكاريوس حنا
المحافظة : الجيزة
الحى / المدينة : امبابه
رقم البطاقة : 26903272102129 رقم التليفون : 01224711159
العنوان : البصراوي
العمر (تاريخ الميلاد) : 27-03-1969 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 05-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبتات
الاروماتيز [14161]
قسم العلاج :
الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: إيمان فهد مكاريس حنا National ID: 26903272102129	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بمكتب الأوراق الشخصية

أيمان
فهم مكاربوس حنا
٦ ش فؤاد الشيخ ش البصراوي
امياية - الجيزة

١٩٥٩/١٠

٢ ٦٩ . ٣ ٢٧ ٢١ . ٢١ ٢٩

KM5928341

١٠٢٥/٠٤ ٢٠٩٠٣١٧٢١٠٢١٣٩

حاصلة على ديوم تجارة

أنش مسيحية متروجة
عنبر ابراهيم و هبة ابراهيم

البطاقة سارية حتى ١٠٣١/٠٤/٠٦

Patient Name : ايمان فهد مكاربوس حنا
Patient Info. : Female 56Y
Department : Out Patient
Profile NO. : 1165-2024

Patient Id : 11796
VisitId : 110393
Collection Date : 03/01/2026 15:23:30
Reporting Date : 03/01/2026 15:41:11

Clinical Chemistry Report

<u>Test</u>	<u>Result</u>	<u>Unit</u>	<u>Reference Range</u>
<u>Kidney Function</u>			
Calcium Total	: H 10.25	mg/dL	8.46 - 10.1

Signature
Dr. Manal Mokhtar

Name: ايمان فاهيم مكاريوس حنا		
MR. : 7948		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه The patient presents to the daycare clinic for initiation of Zometa and Femara therapy along with supportive treatment. Laboratory results have been accepted and reviewed. The plan is to commence Zometa and Femara as indicated, with appropriate supportive measures to be implemented.		
تحريراً في ,		
التاريخ :	الوقت :	20-09-2026
الطبيب المعالج :	هنا اسامه عبد الرحيم عبد البر	
يعتمد ,		

07/10/2024, 10:10

code:42 -- Name_A MDT form Harmal -- Name_E MDT form Harmal

Presidential Program for Women's Health
Ministry of Health and Populationالبرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:

1- Name : ايمان فهد مكيروس حنا

Sex:

3- Medical numbers:

Female

83199854

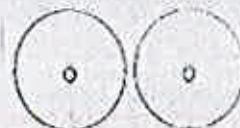
Old Med:

Old Med:

1165-2024

2-ID number:

Patient Age : 56 Y



Family history

Family History
FvB

Phone

PHONE

01224711159

BMI

BMI

37

REASON OF HDT

Reason of HDT

marital status

Marital Status

NRS Score

NRS Score

2

GROUP

GROUP

Date of HDT

Date of HDT
22/10/2024

Co morbidity

Co morbidity

hln,hysterectomy
status

Menopausal Status

Initial STAGE

Stage STAGE

Clinical data

Breast Imaging :

rt breast 10 o'clock 2 x1 cms intramammary LN+axillary LN non sp.

Mets Workup :

ct pet scan:free

BX

ER

PR

Her 2 Neu

SISH

Ki67

IDC

E/S

4/8

2

Amplified

PR COMMENT:

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT:

T

N

M

2

1

0

Sonomammography

PATIENT HISTORY

pt has allergy from contrast

last mdt 1/5/2024: pt will be for NACT TCHP X 6 +surgical decision after mammo

pt ended TCHP x 6 ended 29/8/2024

RT WLE+SLNB:free margins

LN 0/4

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

In view of non pCR status post NACT , for:

1- PORT to Rt Breast, CW & PCLN

2- Adjuvant Kadcyla (TDM1) for 10 injections.

3- Adjuvant ET (consider AI after hormonal profile)

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Patient Name: Eman Fahim Macarios Hanna	Date : 05/09/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 55 Year(s)
Medical ID: 1165/2024	Pat. ID: 53417

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast pathologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UOQ clip
- Complementary ultrasound revealed right breast subtle area of altered parenchyma, seen at 11o'clock measured 0.6x0.7cm, 1cm from the skin, 6cm from the nipple (was 1.2x1.2cm, in prior study dated 2/5/2024)
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific left axillary LN
- The previously reported right axillary LN, is no more appreciated in today's study
- Free right infra and supra-clavicular regions.

Opinion:

- Partial radiological response of the right breast UOQ malignant mass (BIRADS 6)
- Complete radiological response of the right axillary LN
- Benign findings left breast (BIRADS 2)

Yours truly

Signature Dr. Engy Abdallah

05/09/2024 1:51 PM

Patent No.	26903272102129
Pat. No.	SR-480/24
Sex	Female
Age	55 Y
Referring Doc	

26903272102126

Path no. SR-480/24

SR-480
Female

Age = 55 Y

Referring Use:

Request No. : 13001585

Request Date : 17/04/2024

Verification Date : 22/04/2024

Financial Category : المصارف - نفقات

Department : Out Patient

Registered by: Umarwa mohamed youssa

Pathology Report

CLINICAL DATA

Flight breast cancer

NATURE OF SPECIMEN

Flow and slides revision

GROSS PATHOLOGY

One paraffin block labeled 200483/24 and 9 slides

HISTOPATHOLOGY

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: No
- * Perineural invasion: No
- * Angiolymphatic invasion: No
- * Right axillary lymph node: metastatic deposits.
- * IHC (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Weak nuclear reaction in 15% of tumor cells (score 4/8).
- Her2: Moderate complete membrane reaction in more than 10% of tumor cells (score 2+).
- Her2 by FISH: Amplified (showing small clusters).

- Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+1) Grade II

- Oral carcinoma in situ: No

- Perineural invasion: No

- Angiogenesis: No

- * Right axillary lymph node: metastatic deposits

- ER: Strong nuclear reaction in both

- PR: Weak nuclear reaction in 15% of tumor cells (score 1/3)

- PR: Weak nuclear reaction in 15% of tumor cells (score 8/8)

- Her2 by SISH: Amplified (showing small clusters)

- Amplified (showing small clusters)

DIAGNOSIS

Right breast cancer, Slide revision (Right breast tru cut biopsy and FNAC from right axillary lymph node)
Invasive duct carcinoma, Grade II

Invasive duct carcinoma, Grade II
Nodular architecture

Non-axillary LN: Metastatic carcinoma consistent with breast origin

Verified by:

Dr Mohamed Torky

Printing Date: 21-04-2024 08:32:00

Page 1 of 2

Printing By : marwa mohamed youss

Patient Name : (المستند / السيد محمد محمد يوسف حسن)
Patient No : 26903272102129
Path No : SR-480/24
Sex : Female
Age : 55 Y
Ref: Dr. Goc

Request No : 13001585
Request Date : 17/04/2024
Verification Date : 22/04/2024
Financial Category : وثيقة التأمين - وثيقة التأمين
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
- Her2 by SISH: amplified with small clusters.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

SIGNATURE

Dr. Rana Fathy
Dr. Muhammad Torkey

Verified by :
Dr. Mohamed Torkey

Printing Date : 22/04/2024 08:32:00

Page 1 of 2

Printing By : marwa mohamed youss

Dr
Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr
Shady Elia Anis

M. Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard Medical School, USA
Member, United States Canadian Academy of Pathology

Patient Name : ایمان فہیم مکاریوس حنا

Age : 55 Y

Referred By Prof. Dr. :

Clinical Diagnosis : Right breast mass

Nature of Specimen : Referred one block labelled as 200483/24 (Core biopsy)

Receiving Date : 2024-04-07

Analysis ID :  84

Gender : Female

Patient ID : 634176

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections prepared from the paraffin block were stained for HER 2 gene and chromosome 17 using Silver In Situ Hybridization (SISH) technique in Benchmark XT (Ventana).

Results :

The ratio of Her 2 signals to chromosome 17 signals in the nuclei of the tumour cells is 3 (more than 2) indicating amplification of HER2 gene.

Average Her 2 copy nb/cell is 6

Conclusion :

Referred block, HER 2 by SISH, POSITIVE HER 2 GENE AMPLIFICATION, RATIO = 3

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak



This report is electronically signed, any alteration or modification is void.

Identikit

Patient Name: Aman Fahim Makariosa

Patient ID: 630885

Exam Date: 8/1/2024

Whole Body PET/CT Examination

History and Clinical Data:-

Patient data:-

Female patient, 55 years old, Weight: 100 Kg, Height: 158 cm, Blood Glucose level: 101 mg/dl

Radiopharmaceutical:-

F-18-FDG 9 mCi injected I.V.

Clinical background:-

- History of suspicious right breast mass (3/2024) post right axillary lymph node biopsy and pathology revealed adenocarcinoma of breast origin.

Purpose of the study:-

- Initial evaluation.

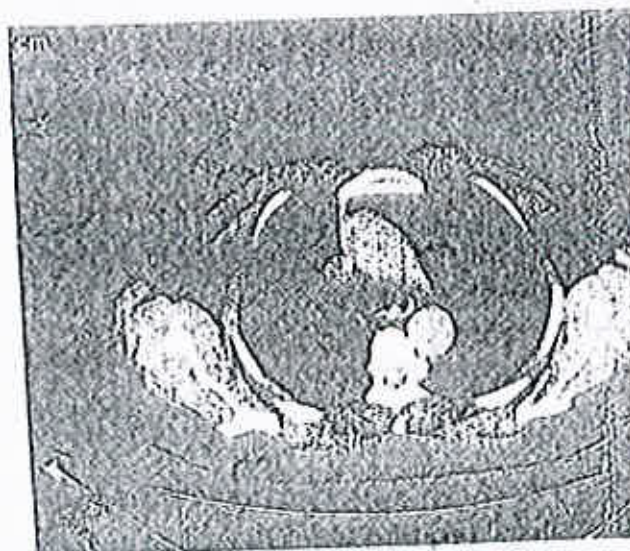
Technique:-

- Approximately one hour following the IV injection of the radiopharmaceutical, a total body PET-CT was acquired from the knees to the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~ 2.86.
- Mediastinal blood pool average SUV ~ 2.2.

Findings:-

Breasts and Axillary regions:-

- Low grade metabolically active right breast upper outer quadrant soft tissue nodule measuring about 1.8 x 1.2 cm with max SUV ~ 1.35.



Patient Name: Iman Fahim Makariosu

Patient ID: 650885

Exam Date: 8/4/2024

- No metabolically active left breast masses.
- Few low grade metabolically active right axillary lymph nodes measuring up to 1.5 cm with max SUV ~ 1.11.
- No metabolically active left axillary lymph nodes.

Head and Neck:-

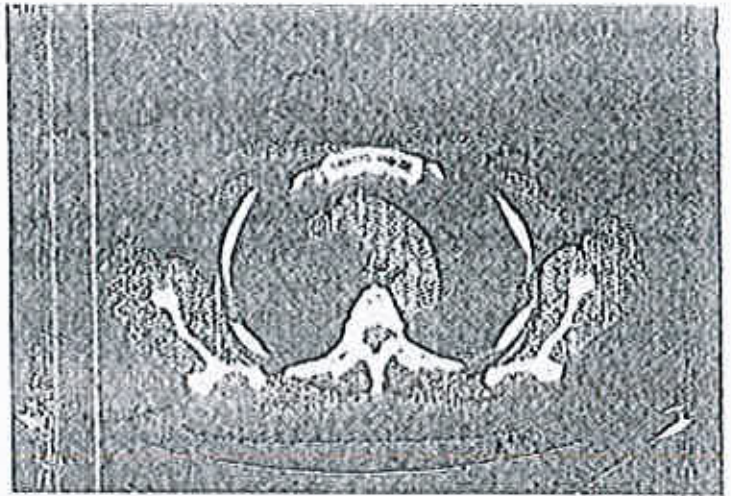
- Scanned brain sections show uniform bio-tracer distribution in the cerebral and cerebellar hemispheres.
- No metabolically active (FDG avid) masses are noted.
- No metabolically active (FDG avid) cervical lymph-nodes are noted.

Upper Chest:-

- No metabolically active (FDG avid) pulmonary nodules or masses are noted.
- No metabolically active (FDG avid) pleural effusion is noted.
- No FDG avid (metabolically active) mediastinal lymph-nodes are noted.
- Aberrant right subclavian artery (normal variant)
- Borderline cardiac size with no pericardial effusion.
- Low grade metabolically active bilateral elastofibroma dorsi.

Abdomen and pelvis:-

- Average sized liver showing no metabolically active (FDG avid) focal lesions.
- Average sized spleen showing no metabolically active (FDG avid) focal lesions.
- No metabolically active (FDG avid) porta hepatis, mesenteric, retroperitoneal or pelvic lymph-nodes are noted.
- No ascites.
- Evidence of subtotal hysterectomy with clear operative bed.



Patient Name: *Iman Fahim Makariosu*

Patient ID: *650885*

Exam Date: *8/4/2021*

- The rest of the abdominal and pelvic organs show normal PET/CT appearance with no related focal lesions or abnormal FDG uptake.

1) Axial and scanned Appendicular Skeleton:-

- No (FDG avid) metabolically active osteolytic or osteosclerotic lesions are noted.
- Diffusely increased FDG uptake along scanned skeleton with max SUV ~ 4.25 (likely anemia related).
- Bone island of the left femoral head.

Conclusion:

- *Positive study for low grade metabolically active right breast soft tissue nodule with regional low grade metabolically active nodal metastatic deposits.*
- *No metabolically active distant metastatic deposits.*
- *Stage IIa (T1, N1, M0) right breast cancer.*

Thank You for Your reference

PET/CT Unit

Dr. Mohamed Aboregela, Msc

Revised by Dr / ph

Typed by : Aouf Hoss

Radiology consultant

Prof. Dr. Ramy Edward, M.D